

2 June 2023

Lucy Letby - Cross-examination Day 6: Babies I, J & K

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MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

Friday, 2 June 2023

(1.00 pm)

(In the presence of the jury)

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

MR JUSTICE GOSS: Thank you all, members of the jury, very much for making yourselves available to continue with the trial today. As I said to you, we will have a reasonable session now, then we'll have a slightly longer break this afternoon, then finish at around the usual time, certainly by 4 o'clock. All right?

Thank you very much.

Yes, Mr Johnson.

MR JOHNSON: Thank you, my Lord.

It has been, I think, 8 days since you were last sitting there. In those days have you thought of anything that you have said so far that you would like to alter or review?

LETBY: No.

JOHNSON: I would like to move on, if we may, to the third incident involving [Baby I]. All right?

LETBY: Yes.

JOHNSON: This happened towards the end of the night shift of the 13th into 14 October 2015. All right?

LETBY: Okay.

JOHNSON: Have you got your defence statement there? I think it's just there under the iPad. You deal with [Baby I] at page 14. You can see that at paragraph 113 you deal with 30 September. At paragraph 115 on page 15 you deal with the incident that we have

just dealt with, which is what happened in nursery 2 in the early hours of 13 October.

With me so far?

LETBY: Yes.

JOHNSON: And you don't mention at all -- before you get to -- at paragraph 119, which is [Baby I]'s final collapse and death, you don't mention at all the incident on 14 October.

LETBY: No.

JOHNSON: When you were asked questions by your counsel on 15 May, you said that you had no recollection of what happened on this shift or of [Baby I]'s desaturation; do you remember saying that?

LETBY: Yes.

JOHNSON: Was that the truth?

LETBY: Yes.

JOHNSON: Is it your case that the staffing levels on this shift caused or contributed to the life-threatening desaturation suffered by [Baby I] on 14 October?

LETBY: No.

JOHNSON: Is it your case that medical competence or incompetence contributed in any way?

LETBY: No.

JOHNSON: Is it your case that anybody's mistakes made a contribution to [Baby I]'s desaturation?

LETBY: No.

JOHNSON: If we go to the [Baby I] sequence, please, Mr Murphy, it's sequence number 3, and could we go to tile 7. Click on that, please.

This gives us the layout, doesn't it? Can you see that?

LETBY: Yes.

JOHNSON: You see that you were one-to-one with [Baby I]?

LETBY: Yes.

JOHNSON: Joanne Williams, with two other children, that were also in nursery 1?

LETBY: Yes.

JOHNSON: Nursery 2, Laura Eagles looking after two, who included [Baby G]?

LETBY: Yes.

JOHNSON: Janet Cox in nursery 4 with a single child?

LETBY: Yes.

JOHNSON: And then some other babies whose location cannot be confirmed?

LETBY: Yes.

JOHNSON: Should we take it though, that given that Janet Cox was looking after three of those other babies, they are likely to have been either in nursery 3 or nursery 4?

LETBY: Yes.

JOHNSON: Is there any inference we can draw, so far as the other three babies are concerned, who were being looked after by Caroline Oakley and Laura Eagles?

LETBY: No, I couldn't say.

JOHNSON: Okay. If we go to tile 46, please. This concerns an examination by Dr Neame very early in the shift at 22.05.

LETBY: Yes.

JOHNSON: It shows that [Baby I] was stable, pink, settled and had good chest movement, albeit she was on a ventilator. Do you remember that?

LETBY: I don't remember the shift specifically, but yes, I agree that's written there, yes.

JOHNSON: Yes. Well, you've heard the evidence relating to all this, haven't you?

LETBY: Yes.

JOHNSON: Do you agree that by 1 o'clock the following morning on 14 October, [Baby I] was improving?

LETBY: I'd have to review the notes again. I can't say from my memory.

JOHNSON: We'll look at your note then at tile 60, please. We can see at the top -- this is the note that you made towards the beginning of the shift; is that right?

LETBY: Yes.

JOHNSON: If we scroll down, please, do we see a note that you made at the end of the shift at 08.43?

LETBY: Yes.

JOHNSON: We can see that you have recorded the fact that the blood gas at 01.00, which is why I have selected that time, was good?

LETBY: Yes.

JOHNSON: No changes had been made to the ventilator?

LETBY: No.

JOHNSON: Temperature elevated, incubator temperature reduced; is that right?

LETBY: Yes.

JOHNSON: And other signs were good at that stage, would you agree?

LETBY: Yes.

JOHNSON: So, for example:

"[Baby I] tolerating handling better. Tone appears improved. Remains pale. Abdomen distended but soft. Nil obtained from the NG tube"?

LETBY: Yes.

JOHNSON: And you have then made a note, haven't you, of events at 05.00 hours?

LETBY: Yes.

JOHNSON: Just tell us what you have noted there.

LETBY: "At 05.00, abdomen noted to be more distended and firmer in appearance with area of discolouration spreading on right-hand side. Veins more prominent. Oxygen requirement began to increase, colour became pale. Reviewed by Reg Neame. Ventilation increased to CMV, rate 70, 30/6, gradually requiring 100% oxygen. Blood gases poor as charted. Clear air entry, slightly reduced on left. Chest movement reduced. Nil obtained from NG tube throughout. Continued to decline. Re-intubated at approximately 07.00. Initially responded well. Consultant Jayaram aware and on way to unit. Abdomen firm and distended. Overall colour pale. X-rays carried out. Resuscitation commenced as documented in medical notes. Night and day staff members present. Drugs as documented given via peripheral line. Ciprofloxacin stopped to allow access therefore only half dose completed. Morphine increased to 40 micrograms per kilogram per hour and dobutamine increased to 20 micrograms per kilo per [hour]. [Baby I] placed back onto ventilator and care handed over to Staff Nurse Tomlins [as read]."

JOHNSON: Please describe for us the discolouration that you saw.

LETBY: I can't recall that from my memory now.

JOHNSON: But it was moving discolouration?

LETBY: No, I don't believe it was moving.

JOHNSON: Well, what does the word "spreading" mean?

LETBY: That the area was getting bigger, not that it was actively moving. The discolouration was becoming bigger in size.

JOHNSON: So do you remember it?

LETBY: No, but that's how I read my own note to be.

JOHNSON: Where did you get the time of 5 o'clock from?

LETBY: I don't know.

JOHNSON: Had you ever seen this sort of spreading discolouration before?

LETBY: I can't comment because I don't remember it right now.

JOHNSON: Don't or won't?

LETBY: No, I don't remember it.

JOHNSON: Where did you get the time of 5 o'clock from?

LETBY: I don't know. I don't know if it's from paper charts, I'm not sure from my memory.

JOHNSON: Well, I'm going to suggest to you that if you really had seen this at 05.00 hours, as you have recorded, you would have escalated it straightaway. Do you agree with that or not?

LETBY: I can't comment on what time the doctor did come. It does say I've asked for a review. I'm not sure without looking at the notes when that happened.

JOHNSON: What had happened about 24 hours earlier?

LETBY: I don't know what you mean, sorry. With [Baby I]?

JOHNSON: I think you know who I'm talking about, don't you?

LETBY: I don't understand the question that you're asking me.

JOHNSON: What had happened to [Baby I] about 24 hours earlier?

LETBY: She'd been found in the cot.

JOHNSON: In what state?

LETBY: Apnoeic.

JOHNSON: Oh, come on. She wasn't apnoeic --

LETBY: Gasping.

JOHNSON: Almost dead. Do you agree?

LETBY: Yes.

JOHNSON: Yes. So this child, who had been almost dead 24 hours earlier, according to your note, has now got a more distended and firm abdomen. Is that a positive or a negative sign?

LETBY: Negative.

JOHNSON: She has discolouration which is spreading on her right-hand side. Is that a positive or negative sign?

LETBY: Negative.

JOHNSON: So what would you have done if what you wrote was true?

LETBY: I would have escalated it to somebody, either a senior nurse or a doctor.

JOHNSON: But we know what time Dr Neame attended, don't we?

LETBY: No, I don't know from my memory now.

JOHNSON: Let's go to tile 79:

"Asked to see patient re increased requirement for oxygen..."

Is that right?

LETBY: Yes.

JOHNSON: "... and decreased saturations"?

LETBY: Yes.

JOHNSON: Where is there mention to the doctor of discolouration?

LETBY: I can't see that there until further down into the text.

JOHNSON: Well, where do you see it?

LETBY: "Abdomen distended and mottled."

JOHNSON: Any evidence of moving discolouration?

LETBY: No.

JOHNSON: Whose responsibility was it to communicate to Dr Neame the issue that you have recorded in your notes as having occurred at 5 o'clock?

LETBY: Mine.

JOHNSON: You didn't communicate it, did you?

LETBY: I can't say specifically what time I asked Dr Neame to come. He obviously has come at 5.55, yes.

JOHNSON: Is what you have recorded in your nursing note a serious event?

LETBY: It's showing a decline, yes.

JOHNSON: Oh, but let's look at the context of this decline.

A child that had almost died 24 hours earlier; correct?

LETBY: Yes.

JOHNSON: "Abdomen more distended and firmer. Area of discolouration spreading on right-hand side. Veins more prominent. Oxygen requirement beginning to increase.

Colour became pale."

All your note?

LETBY: Yes.

JOHNSON: This would have been an emergency, wouldn't it, if what you wrote was true?

LETBY: I don't believe it was an emergency. It was showing a decline, yes, and I'm not sure who I escalated it to and when.

JOHNSON: This is another example, isn't it, Lucy Letby, of you writing a nursing note that creates the impression of a more gradual decline than in fact really happened?

LETBY: No, I don't agree.

JOHNSON: You sabotaged [Baby I] at about 6 o'clock, didn't you?

LETBY: No.

JOHNSON: I'm going to suggest to you that Dr Neame was called at about 5.50; okay?

LETBY: Okay.

JOHNSON: If we go to page 78, I'll show you the reason why and you can comment. If we go back to tile 78, please.

There we can see, in the third column of infusions, an infusion of morphine sulphate being given to [Baby I] at about 5.50, prescribed by Dr Neame --

LETBY: Yes.

JOHNSON: -- with your signature underneath.

LETBY: Yes.

JOHNSON: And this goes with his note, doesn't it, that I've just shown you?

LETBY: Yes.

JOHNSON: And this is the time he arrived, isn't it?

LETBY: From that, I can't say what time he prescribed it, but yes, the infusion was commenced at 5.50, so it would have been prescribed prior to that, yes.

JOHNSON: Yes. Go back to tile 75 though, please -- sorry, tile 76, I beg your pardon. Down to the bottom, please.

I'm afraid this is one of the ones where we may have to select part of the -- thank you. Scroll up, please.

We see your initials, don't we, at 01.00 hours, down to 07.00 hours?

LETBY: Yes.

JOHNSON: We also see under the 5 o'clock column your handwriting, don't we --

LETBY: Yes.

JOHNSON: -- in that large left-hand column? Does it read "metronidazole"?

LETBY: Yes.

JOHNSON: Does it also read:

"05.30, abdo distended ++"?

LETBY: Yes.

JOHNSON: Why is there such a difference between your nursing note and what you have written in handwriting there?

LETBY: The nursing note was reflecting a period from 5 o'clock when her observations started to change.

JOHNSON: What was it about the observations that started to change at 5 o'clock?

LETBY: She was needing an increase in oxygen and her abdomen was obviously becoming distended at about 05.30.

JOHNSON: Where do we see the increase in oxygen on this document?

LETBY: On the chart there she's gone from 60% to, between 5 and 6, 100%.

JOHNSON: Where is that, sorry?

LETBY: On the FiO2 column. I can't... Does the mouse...?

JOHNSON: Yes.

LETBY: That doesn't work, sorry.

JOHNSON: All right.

LETBY: It's on the left-hand column where the two ticks are.

JOHNSON: Where it goes to 100%?

LETBY: Yes.

JOHNSON: Which column is that in?

LETBY: 6 o'clock.

JOHNSON: Yes, not the 5 o'clock column, is it?

LETBY: No. But by 6 o'clock, that was her oxygen requirement.

JOHNSON: Can you see -- just where the cursor is now, do you see it, where it's pointing to a 60?

LETBY: Yes.

JOHNSON: Can you see that you have written in something above that?

LETBY: "Squeaky", yes.

JOHNSON: Yes. What does "squeaky" mean?

LETBY: Air entry, so the ticks there are for left and right-sided chest movement and air entry and I've said it's squeaky and that's led me to then do suction.

JOHNSON: What does squeaky mean?

LETBY: Squeaky means you haven't got clear -- when doctors and nurses write "clear entry", you don't hear anything, but to hear a squeak means you're not getting full lung expansion.

JOHNSON: Let's look at that then. At 5 o'clock, according to you, you've got a child who was almost dead 24 hours earlier; correct?

LETBY: Yes.

JOHNSON: A child whose abdomen has expanded and is becoming harder; yes?

LETBY: Yes, at 05.30, yes.

JOHNSON: 5 o'clock according to your nursing note, but okay, 5.30 here. A child with moving discolouration or spreading discolouration; yes?

LETBY: Yes.

JOHNSON: Who's on a ventilator and has got squeaky air entry?

LETBY: Yes.

JOHNSON: And you didn't escalate it?

LETBY: Squeaky air entry is not an emergency. The next thing we were going to do is suction, which is what I've gone on to do. We wouldn't be putting out an emergency call for a squeaky air entry.

JOHNSON: But were not restricting ourselves to a squeaky air entry on what you have recorded, are we?

LETBY: No.

JOHNSON: We've got an expanding hardening abdomen and expanding discolouration.

LETBY: Yes. I can't say now at what time I've bleeped the doctors, I don't know, or who I escalated it to.

JOHNSON: "Asked to see patient" isn't a very urgent request, is it? It's not crash called or bleeped.

LETBY: It's a bleep, yes, because there's no other way of asking the doctor to review them. I don't think that warranted a crash call, no.

JOHNSON: Just think about that: are you seriously suggesting that other than a bleep there is no way to ask a doctor a question or to review a patient?

LETBY: Unless they're on the unit.

JOHNSON: Well, let's think back to [Baby E] and your evidence about [Baby E], the mucky aspirate.

LETBY: Yes.

JOHNSON: What did you tell the jury was the way of communicating with the doctor on that occasion?

LETBY: Telephone.

JOHNSON: Yes.

LETBY: But the telephone comes from a bleep. I can't -- the only way nurses can speak to a doctor is by bleeping them first. There's no way I can just pick up the phone and say, "Can I speak to..." It has to go through the bleep system.

JOHNSON: Why don't you mention the discolouration on this manuscript note?

LETBY: I don't know, I can't answer that.

JOHNSON: You took the word squeaky from Dr Neame's note at 5.55, didn't you?

LETBY: I don't agree, no.

JOHNSON: Let's look at his note. Go back to his note, please, Mr Murphy, it's tile 79:

"On my arrival Neopuffing. Some chest wall movement. Positive capnograph change. Equal air entry. Some squeaky sounds."

LETBY: Yes.

JOHNSON: So like you copied Dr Beebe's note in relation -- on 30 September, from an examination at 11.40, here you have done the same, haven't you?

LETBY: No, I disagree.

JOHNSON: Do you remember Dr Neame saying that the mottling he saw, that he has recorded there, "Abdomen distended and mottled", was unusual?

LETBY: Yes.

JOHNSON: Did you see anything unusual?

LETBY: I can't recall specifically.

JOHNSON: Go to tile 85 and go to the report of Dr Wright, please.

Do you see that there Dr Wright has recorded the fact that the degree of gaseous distension of the bowel has increased since the previous image of 13 October?

LETBY: Yes.

JOHNSON: [Baby I] had been on a ventilator since then, hadn't she?

LETBY: Yes.

JOHNSON: And the tube goes directly into the lungs, so it doesn't go into the stomach or via the stomach into the bowel, does it?

LETBY: No.

JOHNSON: That's because you had injected [Baby I] with air, isn't it?

LETBY: No.

JOHNSON: [Baby I] had been nil by mouth, hadn't she?

LETBY: Yes.

JOHNSON: By the end of your shift, if we go to tile 129, everything had been decompressed, hadn't it?

Sorry, I've misread it. It says:

"No significant change in the degree of bowel distension since the previous image earlier in the day."

LETBY: Yes.

JOHNSON: You had inflated [Baby I] with air, hadn't you, Lucy Letby?

LETBY: No.

JOHNSON: [Baby I] collapsed again at 7 o'clock, didn't she?

LETBY: Yes.

JOHNSON: This is one of those cases when air was going in and out of her, but she was not oxygenating; do you remember that?

LETBY: Yes.

JOHNSON: If we look at your -- so that's at 7 o'clock, and if anybody needs the reference for that it's in Dr Neame's note at tile 106. If we see your note, it's at 105, please.

I think you read us through it before. Just to remind you, we've got that final paragraph and just above it, it says, about three lines up:

"Re-intubated at approximately 7 o'clock."

Have you got that?

LETBY: Yes.

JOHNSON: And by this stage, "Abdomen firm and distended"?

LETBY: Yes.

JOHNSON: That is because you were sabotaging her, isn't it?

LETBY: No.

JOHNSON: Look at tile 130, please. You record the resuscitation with both the night staff and the day staff present; is that right?

LETBY: Yes.

JOHNSON: The drugs being given to [Baby I]?

LETBY: Yes.

JOHNSON: And the fact that care was handed over to Shelley Tomlins?

LETBY: Yes.

JOHNSON: And none of that you now remember?

LETBY: Not with clear detail, no.

JOHNSON: Well, let's move on to [Baby I]'s death and the night shift of the 22nd to 23 October.

I'm going to go back 24 hours to -- well, let's just remind you and remind the jury of what happened to [Baby I] after the collapse we've just dealt with.

Between 15 October and the 17th she was taken to Arrowe Park; do you remember that evidence?

LETBY: Yes.

JOHNSON: On 17 October she was returned to Chester and she made good progress thereafter and I'm going to take up the chronology on the night shift of the 21st into the 22nd; all right?

LETBY: Yes.

JOHNSON: So in other words, 24 hours before you were looking after her when she died -- well, 24 hours before she died, I should say. All right?

Do you accept the evidence of Ashleigh Hudson that on that night shift, the 21st to the 22nd, [Baby I] was easy to settle?

LETBY: Yes.

JOHNSON: Do you accept that [Baby I]'s observations were stable with the slight exception of a single raised respiration reading at 22.00 hours on that shift?

LETBY: I can't say that now without seeing the chart. I can't remember all of the observations, sorry.

JOHNSON: All right. If you want to look at it, by all means. If you look in jury bundle number 2, the one with the big blue 2 on, and if we go to divider 12, if we start at the back of divider 12 with the documents that have a 4 in blue numbering in the right-hand corner and you go to page J15034, it should be the first of the documents with the blue number 2.

LETBY: It's a 4?

JOHNSON: It's got a 4 in the top right-hand corner and in the bottom right-hand corner it's got J15034.

LETBY: Yes.

JOHNSON: Here we have [Baby I]'s observations for the 20th, the 21st and the 22nd -- or the morning of 22 October; do you see that?

LETBY: Yes.

JOHNSON: So this covers the night shift of the 21st into 22 October, which is what I'm asking you about at the moment.

LETBY: Yes.

JOHNSON: Just to remind you, to put that into context, that is 24 hours before [Baby I]'s death.

LETBY: Yes.

JOHNSON: Okay. The suggestion I was making to you, and inviting you to confirm, was that [Baby I]'s observations were stable on that shift, with the slight exception of a single raised respiration reading at 22.00 hours.

LETBY: Yes.

JOHNSON: So we're agreed about that?

LETBY: Yes.

JOHNSON: Ashleigh Hudson told us that [Baby I] was being kept in nursery 1 as a precaution, but she wasn't an ITU baby; do you agree with that?

LETBY: Yes.

JOHNSON: Do you agree from this chart that she was self-ventilating in air and that her saturations were optimal?

LETBY: Yes.

JOHNSON: Do you agree that she had a non-distended abdomen?

LETBY: Again, I can't say from looking at any of this, but if that's what they stated then, yes.

JOHNSON: Just to satisfy you that I'm not misleading you, if we go to [Baby I] 4, please, and T9. We can go back to the original behind this if you want to, but if you look about five lines up from the bottom of the page it says that:

"[The] tube was on free drainage, [there were] minimal aspirates, clear fluid, abdomen soft and non-distended."

LETBY: Yes.

JOHNSON: Have you got that?

LETBY: Yes, thank you.

JOHNSON: So that was the position on the night shift according to Ashleigh Hudson.

LETBY: Yes.

JOHNSON: Caroline Oakley took over at 8 am, or thereabouts, on 22 October. Her evidence was read to the jury by agreement. She

told the jury that [Baby I] was alert, that her abdomen was soft and non-distended, that there were minimal aspirates from the NGT. And if we go to tile 23, please, we can see part of her note: [Baby I] had been out for cuddles.

LETBY: Yes.

JOHNSON: So would you agree that despite three life-threatening situations in the preceding 3 weeks, [Baby I] appears to have been in good condition at that time on the day shift of 22 October?

LETBY: Yes.

JOHNSON: So we come to the night shift on which she died. I'd like to start, please, with tile 47. Do we see there that Ashleigh Hudson, again, was looking after [Baby I] together with another child in nursery 1?

LETBY: Yes.

JOHNSON: That Valerie Thomas had a couple of children in transitional care?

LETBY: Yes.

JOHNSON: And you had two children. One was HS --

LETBY: Yes.

JOHNSON: -- in nursery 2. The other, a baby in nursery 3, MC?

LETBY: Yes.

JOHNSON: Finally, do we see that Christopher Booth, the band 6, had [Baby G] also in nursery 3?

LETBY: Yes.

JOHNSON: Just to remind you, HS was a child who went to Stoke Hospital that evening.

LETBY: Right.

JOHNSON: Do you remember the child that I'm talking about?

LETBY: Not with any clarity, no.

JOHNSON: All right. He'd come from Stoke and he went back to Stoke during this shift.

LETBY: Right.

JOHNSON: We will come to that in a moment. But first of all, I'd like to ask you the questions that I have asked you in relation to each of these incidents. Are you suggesting that the staffing levels caused or contributed to the death on this occasion of [Baby I]?

LETBY: I think potentially there were staffing issues that may have affected her. I don't know how but the staffing was not ideal.

JOHNSON: Right. I will guess a reason that you are saying that as being you don't think Ashleigh Hudson was up to the job.

LETBY: No, it was medical.

JOHNSON: All right. What was the staffing medical issue?

LETBY: That the doctor had to be called away to another delivery over on the labour suite and left [Baby I]. And also Dr Gibbs took her off the ventilator, went home, and within half an hour had to come back again.

JOHNSON: We'll come to that. Is this Dr Chang that you're talking about?

LETBY: Yes, and Dr Gibbs, yes.

JOHNSON: Dr Gibbs and Dr Chang. We'll come back to that.

So far as the nursing staff was concerned, is there any issue here on what we can see on the screen with care?

LETBY: No.

JOHNSON: So the medical issue, the doctor issue, you say was that Dr Chang was called away? Is that it?

LETBY: Yes, I think considering what [Baby I] had been through, as you've just been saying, she was a poorly baby, the doctors weren't with her at all times, no.

JOHNSON: So a child who had been very well for the preceding two shifts you're suggesting, what, should have had a doctor there throughout?

LETBY: Once she deteriorated, yes.

JOHNSON: What about Ashleigh Hudson? Do you accept that she was competent to look after [Baby I] on this shift?

LETBY: Yes. At this time she wasn't needing any specialised care.

JOHNSON: You didn't like being in nursery 3, though, did you?

LETBY: No, I disagree.

JOHNSON: Were you jealous that Ashleigh was in nursery 1 when you were in nursery 3?

LETBY: No, I have nothing to be jealous of.

JOHNSON: Could we look at paragraph 116 of your defence statement, please. What does it say?

LETBY: "Ashleigh had been on the unit since February, she was quite inexperienced to be looking after [Baby I]."

JOHNSON: Yes.

LETBY: This is with reference to 13 October.

JOHNSON: Oh, it is, which is, as you correctly observe, a full 9 days earlier.

LETBY: Yes.

JOHNSON: Had Ashleigh's experience increased significantly in those 9 days?

LETBY: No. But [Baby I] was in nursery 1 at this point, being very closely monitored, so I think Ashleigh was okay to look after her.

JOHNSON: What we're going to need to do now, please, is to open up the neonatal review before this shift. It's in the white --

LETBY: Is there any of this that I can put aside?

JOHNSON: Yes, of course, I'm sorry. Put the defence statement to one side, close up the file with the observation charts in.

(Pause)

You've got the right neonatal review file there.

What I'd like you to do, please, is to go to the very final review in that white file. That's it. On the front of it, it should say "Event 14: [Baby I]".

LETBY: Yes. Thank you.

JOHNSON: If we go to the second page, what we've got is the population distribution that we've just been through at tile 47 of the carousel sequence.

LETBY: Yes.

JOHNSON: HS is the child I was suggesting to you went to Stoke.

Just keeping that there, I'm going to ask Mr Murphy just to put up tile 57 from the [Baby I] number 4 sequence, please.

So this is a text you were sending to your friend [Nurse E] just after the beginning of the shift at 20.47; all right?

LETBY: Yes.

JOHNSON: You were telling [Nurse E] that transport was on its way to take your baby back to Stoke:

"Only 8 babies. Off duty not out."

LETBY: Yes.

JOHNSON: For reasons that we're going to see, the baby going back to Stoke is the baby HS.

LETBY: Okay.

JOHNSON: Do you have any recollection of when HS left?

LETBY: No, I don't, no.

JOHNSON: Well, if we go to line 85 of the sequence -- of the neonatal review, I beg your pardon, do you see there in blue it says at 01.00 hours an entry regarding:

"HS leaving with team at 01.00 hours"?

LETBY: Yes.

JOHNSON: Okay. So if that's the time -- if your note is accurate and that's the time the child was leaving, what time would the transport team have arrived?

LETBY: Some time before that.

JOHNSON: Yes.

LETBY: I couldn't be specific when, but --

JOHNSON: Generally speaking, how long does it take a transport team to get a stable baby ready for transfer?

LETBY: I can't recall this baby specifically to know what needs that it had.

JOHNSON: Well, if he was relatively stable, is it a five-minute job or half an hour or...?

LETBY: Um... I think it depends. I don't think I can comment exactly how long it takes the transport team, but they would have been there a period of time to receive handover and talk to parents, get the baby moved, check any infusions. It's not a five-minute process.

JOHNSON: So it takes a while anyway; is that right?

LETBY: Yes.

JOHNSON: Do you remember being asked to look at this neonatal review by your counsel when you were giving evidence in relation to -- in answer to questions he was asking?

LETBY: Yes.

JOHNSON: Just so you understand, the time at which [Baby I] deteriorated was just before midnight.

LETBY: Right.

JOHNSON: Do you remember that?

LETBY: Yes.

JOHNSON: We can see it marked in the review at line 62, can't we?

LETBY: Yes.

JOHNSON: It's 23.57.

LETBY: Yes.

JOHNSON: So 3 minutes to midnight.

LETBY: Yes.

JOHNSON: The point that you were being asked to deal with was what you were doing in the time leading up to that transfer -- up to that collapse. Do you remember that?

LETBY: Yes.

JOHNSON: So if we go back to page 3 of 8, we can see that there's quite a lot of blue text on that page relating both to the child HS, who is the transfer baby --

LETBY: Yes.

JOHNSON: -- but also to the child MC, who was your other child in nursery 3.

LETBY: Yes.

JOHNSON: But in the hour prior to HS's transfer there doesn't appear to be any entries at all on this neonatal review, does there?

LETBY: No.

JOHNSON: I'm going to suggest that you were dealing with HS at 11 o'clock, 23.00 hours. Can you see that?

LETBY: Yes.

JOHNSON: Do you remember falsifying a record relating to HS?

LETBY: No.

JOHNSON: I'd like to start with a nursing note made at 22.50.

I'll just check I've got the right time. Yes. It's tile 31, please.

Sorry, tile 91 in the [Baby I] number 4 sequence, please.

This is a note made by Ashleigh Hudson. All right?

Can you see there that this is a note made retrospectively at 03.39 but relating to events just before 23.00 hours? Do you see that?

LETBY: Sorry, where does it refer to the time?

JOHNSON: In the black -- where it says Thursday, 22 October, at 22.57.

LETBY: Okay. Sorry, I don't... Where's the 22.57 from?

JOHNSON: I think it was the evidence given by Ashleigh Hudson.

LETBY: Okay.

JOHNSON: So what we see here is:

"[Baby I] was unsettled and rooting at the start of the shift."

Do you see that on the third line?

LETBY: Yes.

JOHNSON: "Settled with dummy and containment. Long line removed due to constant occlusions."

So it was blocked?

LETBY: Yes.

JOHNSON: "Neonatal Nurse Lucy unable to flush so paediatric registrar Rachel Chang informed."

Got that?

LETBY: Yes.

JOHNSON: Now, this comes about, doesn't it, because you, having your QIS qualification, were entitled to access the line, whereas Ashleigh Hudson was not? Is that --

LETBY: That's right, yes.

JOHNSON: So you, because of your more extensive qualification, were the person trusted to have access to [Baby I]'s line at this time?

LETBY: Yes.

JOHNSON: Is that right?

LETBY: Yes.

JOHNSON: I want to look at what else you are noting at this time.

Mr Murphy, could we look at the records of HS, please, and, to start with, J34535.

If we can expand this a little, please, this is Dr Chang's notes at 22.00 hours for HS; okay?

LETBY: Yes.

JOHNSON: Just to put that into context, if we're looking at the neonatal review, this is line 28 in the neonatal review.

LETBY: Yes.

JOHNSON: Have you got that?

LETBY: Yes.

JOHNSON: Where it says:

"Clinical notes about discharge examination. Dr Chang."

LETBY: Yes.

JOHNSON: What Dr Chang has written is:

"Discharge examination. Anterior fontanelle soft."

Then various other physical findings, is that right, all normal --

LETBY: Yes.

JOHNSON: -- without going into detail?

Then the bottom half of the page, please:

"Head circumference. Skin. Long line right leg flushing well. Plan..."

And something about transfer?

LETBY: "Safe for transfer."

JOHNSON: "Safe for transfer", thank you. So that was Dr Chang's examination of your baby at 22.00 hours?

LETBY: Yes.

JOHNSON: 34537 is your nursing note. So this is still HS. Can we expand the second box -- that's it, perfect.

This is a note you made at 22.50?

LETBY: Yes.

JOHNSON: And concluded at 22.52?

LETBY: Yes.

JOHNSON: It says:

"Written for care from 8 pm to present. Emergency equipment checked. Fluids calculated."

Et cetera. Okay? Then it repeats some of the information noted by Dr Chang about 50 minutes earlier or so.

LETBY: Yes.

JOHNSON: Then it says, about five lines down:

"IV fluids, Babiven and lipid via long line. To commence 10% glucose for transfer."

LETBY: Yes.

JOHNSON: What does that involve, what process are you noting here?

LETBY: So TPN and lipid cannot be given to a baby that's undergoing a transfer, so the standard procedure would be that the TPN would stop and a bag of 10% glucose would be put up ready for transfer.

JOHNSON: Yes, so that is what you were about to do, isn't it?

LETBY: Yes, to commence at some point, yes, for transfer.

JOHNSON: "Awaiting arrival of transport team to return to Royal Stoke University Hospital"?

LETBY: Yes.

JOHNSON: That's where I get Stoke from.

That is what you did when you had made that note, isn't it?

LETBY: I can't recall from my memory.

JOHNSON: Well, we'll look at the document. 34542, please.

Is that the note?

LETBY: Yes. I can't see the time though from here.

JOHNSON: Don't worry, we're going to focus on it very closely in a moment. We'll go right in on the time and date in a moment because this is where I'm suggesting you have cooked the records again.

This is the note of the IV fluids being given to the baby HS, isn't it?

LETBY: Yes.

JOHNSON: The one that's referred to in Dr Chang's note and in your nursing note?

LETBY: Yes.

JOHNSON: We see it's 10% dextrose, the volume, the rate and the prescription, which is Dr Chang's signature; is that right?

LETBY: I can't confirm if that's her signature.

JOHNSON: Okay. RC, Rachel Chang, was Dr Chang, wasn't it?

LETBY: Yes, that's RC, yes.

JOHNSON: And your signature and Chris Booth's signature, isn't it?

LETBY: Yes.

JOHNSON: What's the time and date that you have written there?

LETBY: 22/10 at 24.00.

JOHNSON: What was written before? What is a different interpretation of what's written there?

LETBY: That it was 22 and it's changed to a 4.

JOHNSON: A 3 to a 4, 23.00 to 24.00.

LETBY: Okay.

JOHNSON: Well, do you agree or not?

LETBY: I can't say what was beneath it, but it was at 24.00.

JOHNSON: Well, we've just seen, haven't we, that at 10 minutes to 23.00, in other words at 22.50, you were making a note, a timed note, timed by the computer, in the nursing notes that you were to put up dextrose because that's the standard procedure for a transfer.

LETBY: Yes.

JOHNSON: And if this really did say 23.00, this would be 10 minutes after that, wouldn't it?

LETBY: After the note?

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: Yes. That is what was done at 23.00, isn't it, and you have subsequently changed it?

LETBY: No, I would not have changed a record. That was obviously written in error and I've put 24 as the correct time, which Chris would have had to have signed with me.

JOHNSON: Well, that depends on whether the signature is put on before the time, doesn't it?

LETBY: Well, it's part -- when you're signing that, you're signing to say that you agree with the date and time as well.

JOHNSON: Yes, but if you change it after Chris has signed it, he would never know, would he?

LETBY: Well, the charts are there for everybody to look at, but no.

JOHNSON: 23.00 ties in with your timed computerised nursing note, doesn't it?

LETBY: It says at that time, yes, that I will be starting 10% at some point, yes. It doesn't mean that it started at that exact point. It just has to be up for the transfer.

JOHNSON: Well, let's go back to the neonatal review, please, at page 3 of 8. What else were you doing for HS at 23.00 hours? It's line 46.

LETBY: Fluid chart.

JOHNSON: Yes. We'll look at the fluid chart, I think it's 34544, please. What we have here is the fluid balance chart, isn't it?

LETBY: Yes.

JOHNSON: Can I just check what page that was?

(Pause)

I was one page out. 34546.

What we see here is a reading at 23.00 with your initials at the bottom; is that right?

LETBY: Yes.

JOHNSON: And indeed one at 24.00?

LETBY: Yes.

JOHNSON: And this would suggest that TPN was running right up to midnight, wouldn't it?

LETBY: Yes.

JOHNSON: But there's no -- interestingly at 24.00 there's no reading in the pressure VIP line, is there?

LETBY: Yes, there's a zero in both.

JOHNSON: At 24.00?

LETBY: Yes.

JOHNSON: What does it say next to 161? Or is that --

LETBY: Oh, I thought you said the VIP score, sorry. Yes, there's no pressure reading, no.

JOHNSON: Why not?

LETBY: Because the pump's been discontinued.

JOHNSON: You have put those two readings in erroneously, haven't you --

LETBY: No.

JOHNSON: -- because you changed it to dextrose at 23.00?

LETBY: No, I changed it at 24.00.

JOHNSON: And the reason you changed it to 24.00 is because it was at 3 minutes to 24.00 hours that [Baby I] collapsed.

LETBY: No, I disagree.

JOHNSON: Do you remember saying that you had no recollection, when your counsel was asking you questions, of any details other than what is in the notes?

LETBY: For this baby?

JOHNSON: Yes.

LETBY: Yes, I don't remember this baby without the notes, no.

JOHNSON: You said that your memory started from [Baby I] being resuscitated.

LETBY: Yes.

JOHNSON: You said that you went to nursery 1.

LETBY: Yes.

JOHNSON: You believed that Ashleigh alerted you at some point that [Baby I] was unsettled.

LETBY: Yes.

JOHNSON: And that you had had no involvement with [Baby I] before that.

LETBY: From my memory, yes.

JOHNSON: Yes. That's what you were saying in evidence in answer to your --

LETBY: Yes.

JOHNSON: -- own counsel. Can we go to paragraph 120 of your defence statement, though, and see what you were saying back in February 2021 -- sorry, February 2022.

Can you read out paragraph 120, please?

LETBY: "I do not recall a lot of the detail of the night that [Baby I] died. I do remember her crying more than usual and going to her. I called for Ashleigh, who came in. From that point onwards, [Baby I] just deteriorated more and more."

JOHNSON: So what you were saying in your defence statement was that it was you who went to [Baby I] first.

LETBY: No. There were two events: one that Ashleigh called me in for; the other where I believed I was there first.

JOHNSON: Let's deal with that then. So you put me right and explain where I've got it wrong.

LETBY: So in paragraph 119 I say that I do recall Ashleigh calling me in to help but I cannot remember where I was beforehand.

JOHNSON: I see.

LETBY: 120 is referring to a separate incident:

"I called for Ashleigh, who came in."

JOHNSON: Well, I've clearly misunderstood it. Do you remember what Ashleigh said about this desaturation?

LETBY: No. Which one, sorry?

JOHNSON: The one at midnight.

LETBY: Yes, that [Baby I] had been very unsettled and she'd put her prone.

JOHNSON: Do you remember her saying that she heard a cry from [Baby I] that she had not heard before?

LETBY: Yes.

JOHNSON: A loud, relentless, almost constant with no fluctuation cry?

LETBY: Yes.

JOHNSON: Very different to a hunger cry?

LETBY: Yes.

JOHNSON: Is she right about that?

LETBY: I did not hear that cry, no. When I went into the nursery at that point, [Baby I] was quiet.

JOHNSON: Right. So Ashleigh says she's making a sound she'd never heard before, you say [Baby I] was quiet?

LETBY: Yes, Ashleigh called me in because [Baby I] had become quiet and apnoeic. She was not screaming when I went into the room, no.

JOHNSON: Do you remember [Baby I] being intubated by Dr Chang?

LETBY: I know that she was intubated, yes; I can't recall the specific details.

JOHNSON: Do you remember Dr Gibbs saying that both you and Ashleigh Hudson were involved in that at that time with [Baby I]?

LETBY: I don't recall him saying that, but I don't disagree.

JOHNSON: You do disagree or you --

LETBY: I don't disagree. Without looking at the notes, I can't say that I wasn't helping, no.

JOHNSON: It's another case of you gravitating to nursery 1 when you're in one of the less acute nurseries?

LETBY: Well, it's a case of one of the other three members of staff on that night would have had to have assisted.

JOHNSON: Do you remember Dr Gibbs saying that [Baby I] made a very surprising recovery and was breathing against the ventilator and cueing for a feed?

LETBY: I disagree that she was cueing for a feed on a ventilator, I don't think that's physically possible, but yes, she did fight against the ventilator.

JOHNSON: Do you remember Ashleigh Hudson going to call [Baby I]'s parents?

LETBY: No.

JOHNSON: Do you disagree that she did go and call [Baby I]'s parents?

LETBY: No, I just have no independent memory of that.

JOHNSON: Would it be a normal thing for the nursing staff to do?

LETBY: Yes, any member of staff, yes.

JOHNSON: I think you were then involved in giving medication to [Baby I]; do you remember that?

LETBY: Not from my memory, no.

JOHNSON: If we look at tile 117, we may see evidence of that.

LETBY: Yes, I agree, thank you.

JOHNSON: Well, you just explain to us what you agree with and why.

LETBY: I agree it's my signature on the 00.25 row and on the 01.25 and 01.48.

JOHNSON: Do we see a couple more examples of times being changed there?

LETBY: Yes.

JOHNSON: Why is that?

LETBY: I -- I can't explain that. It must be written in error.

JOHNSON: So three mistakes within 25 minutes in the paperwork?

LETBY: Yes, potentially, yes.

JOHNSON: With two different children?

LETBY: Yes.

JOHNSON: And were all just before this child died; is that right?

LETBY: Well, I can't explain why the times have changed, no.

JOHNSON: Well, they've changed because you've changed them.

LETBY: No.

JOHNSON: So who's changed them then?

LETBY: I don't know who has changed them.

MR JUSTICE GOSS: The second one from the bottom, which looks like 01.25, with the 25 overwritten over some other numbers, can you see what the numbers are underneath or do you know what the numbers are underneath?

LETBY: I can't say, no.

MR JOHNSON: Does it say 45 or 48?

(Pause)

That would suggest that things were not done in time order, wouldn't it, that sequence? Do you agree? We go 00.25 or 15, depending on which version we are to accept, we then go 01.38, then 01.25, which appears to have been changed from 01.40 something, and then 01.48.

LETBY: Yes.

JOHNSON: How do those sorts of mistakes happen? You explain to us.

LETBY: That when there's a lot going on on the unit potentially we may have written down the wrong time.

JOHNSON: We or you?

LETBY: Well, I don't believe that that is me. I believe that would have been two of us together.

JOHNSON: So because it's been overwritten twice, you each overwrite it separately, do you?

LETBY: No, but we would both be there for it.

JOHNSON: Like with the 24.00 hours note for HS?

LETBY: Yes.

JOHNSON: Or is it you altering medical records to put some time between yourself and serious events --

LETBY: No --

JOHNSON: -- for [Baby I]?

The time at the top, that's your writing as well, isn't it, the very first time and date?

LETBY: No, I disagree.

JOHNSON: Whose signatures are next to the time?

LETBY: [Nurse C] and Belinda Simcock.

JOHNSON: So you disagree that [Baby I] was crying and her cry was a "loud relentless almost constant with no fluctuation" cry?

LETBY: Yes, that first event, yes.

JOHNSON: Are you saying that the errors in the paperwork are all mistakes made in good faith?

LETBY: Well, I can't say who has overwritten that, whether that was me or anybody else. I have no way of knowing now who did that, but I did not deliberately falsify any paperwork.

JOHNSON: And you can't remember whether Ashleigh Hudson went to phone the parents?

LETBY: No.

JOHNSON: But would it -- can we deal with it this way: would it be normal in these circumstances, for a child in [Baby I]'s position, for the nurse to phone the parents?

LETBY: Yes, it would, it would be a priority for any member of staff to then telephone the parents, yes.

JOHNSON: Do you remember her saying that once [Baby I] had settled, she, Ashleigh, left nursery 1 to maintain the quiet?

LETBY: To what, sorry?

JOHNSON: To maintain quiet in the nursery.

LETBY: I don't recall her saying that, but yes.

JOHNSON: Do you remember telling the jury that you couldn't remember where you were in that hour or so between, if we look at the neonatal review, the event at line 62 and the event at line 86?

LETBY: Yes.

JOHNSON: But we do have, of course, to help us all, material showing when you have either co-signed or made a contribution to records?

LETBY: Yes.

JOHNSON: And so as an example, we see at 00.00 the infusion charts that we've already looked at, line 70, for HS?

LETBY: Yes.

JOHNSON: One just above at 69 for MC?

LETBY: Yes.

JOHNSON: And [Baby I]'s 00.25, which we were just looking at, the altered time on the screen at line 76?

LETBY: Yes.

JOHNSON: We then see two prescriptions for HS which you have signed for. It's actually one event, that, isn't it --

LETBY: Yes.

JOHNSON: -- at 00.46? Yes.

Then we have the time in the nursing note that we looked at earlier, which is your estimate of the time that the care for HS was handed over to the transport team.

LETBY: Yes.

JOHNSON: At 01.06 [Baby I] was crying again, wasn't she?

LETBY: Yes.

JOHNSON: If we go to tile 130, please, in the sequence. Ashleigh has recorded that [Baby I] became unsettled again and it was you who was with [Baby I], wasn't it, at this point when she went in?

LETBY: I can't say 100% that I was there first, but yes, I do remember [Baby I] crying and Ashleigh being with me.

JOHNSON: Well, let's go back to the mistake I made earlier then at paragraph 120 of your defence statement. What do you say there?

LETBY: Yes, that's what I recalled at the time. I couldn't say for definite right now, but yes, I do remember being in the nursery and she was crying.

JOHNSON: And?

LETBY: And that Ashleigh came in.

JOHNSON: No, that's not what you've written, is it?

LETBY: I called for Ashleigh.

JOHNSON: "I called for Ashleigh." That clearly says that you were there first, doesn't it?

LETBY: Um... No, she may have been in the nursery at the time when I've called her over. I couldn't say where Ashleigh exactly was.

JOHNSON: So she may have been in the nursery and come over. What does it say after, "I called for Ashleigh"?

LETBY: "... who came in."

JOHNSON: What, came in from that half of the room to the half of the room you were in?

LETBY: Potentially, yes.

JOHNSON: Sorry?

LETBY: Potentially, yes.

JOHNSON: Potentially?

LETBY: Or in through the door, yes.

JOHNSON: Sorry?

LETBY: She could have come in from the other side of the nursery or she's come in through one of the doors.

I can't be specific.

JOHNSON: Well, do justice to yourself and just think about this before we move on: are you seriously suggesting that, "I called for Ashleigh who came in", doesn't mean that you were in there, Ashleigh was not in there, you called her?

LETBY: I'm saying I don't have any specific memory of where exactly Ashleigh was.

JOHNSON: What were you saying in your defence statement?

LETBY: That I called her and she came in.

JOHNSON: And what did you mean by that?

LETBY: That she came into the nursery.

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: Why wouldn't you just admit that?

LETBY: Because I can't say 100%, sitting here now, that is what happened, so I'm not going to say something is 100% if I can't be definitive.

JOHNSON: Would you have said it in this document if you weren't 100%?

LETBY: No, but this was a period of time ago.

JOHNSON: Right at the end, and we've dealt with this many times, you accept this document as an accurate summary of your case.

LETBY: Yes.

JOHNSON: Yes. That is the truth, isn't it, that you were in there at a time when [Baby I] was crying relentlessly and nobody else was there?

LETBY: Potentially, yes.

JOHNSON: Potentially?

LETBY: Yes, I can't say 100%, no.

JOHNSON: Do you remember Ashleigh saying that when she went in, you had your hands in the incubator?

LETBY: Yes.

JOHNSON: And that's the truth, isn't it?

LETBY: Yes, I do recall trying to settle [Baby I], yes.

JOHNSON: Do you remember Ashleigh saying to you, "She's going to do it again, it's the same cry"?

LETBY: No, I don't recall that conversation word for word, no.

JOHNSON: Well, did she say something like that?

LETBY: She may have done, yes. I can't say specifically.

JOHNSON: And you said, "She just needs to settle"?

LETBY: Yes, my assessment of [Baby I] at that time was that she was hungry and rooting, yes.

JOHNSON: You had pumped [Baby I] with air, hadn't you?

LETBY: No.

JOHNSON: That's why she was so distressed?

LETBY: No.

JOHNSON: Because you were doing your best to kill her?

LETBY: No.

JOHNSON: She hadn't been fed milk for quite some time by this stage, had she?

LETBY: No, that's right.

JOHNSON: And yet her stomach was full of air, wasn't it?

LETBY: After this, yes.

JOHNSON: Yes.

LETBY: She required Neopuff.

JOHNSON: Because you had pumped her full of air, hadn't you?

LETBY: No, I had not, no.

JOHNSON: There then followed a very long attempt to save [Baby I], didn't there?

LETBY: Yes.

JOHNSON: But despite the best efforts of Dr Chang and of Dr Gibbs, she died?

LETBY: Yes.

JOHNSON: The reason for that is because you had injected her circulation with air, isn't it?

LETBY: No.

JOHNSON: There's nothing they could do to prevent her death.

LETBY: No. I have never injected air into any baby.

JOHNSON: Do you remember saying on 16 May that you helped Ashleigh after [Baby I]'s death because this was her, that is Ashleigh's, first death?

LETBY: Yes.

JOHNSON: Do you remember interrupting [Mother of Baby I], [Baby I]'s mum?

LETBY: Interrupting her?

JOHNSON: Yes.

LETBY: No.

JOHNSON: She and her husband were bathing their dead daughter.

LETBY: Yes.

JOHNSON: This was agreed evidence. Do you remember it being read to the jury?

LETBY: Yes.

JOHNSON: And do you remember her saying, these are her words:

"Lucy came back in. She was smiling and kept going on about how she was present at [Baby I]'s first bath and how much [Baby I] had loved it"?

LETBY: Yes.

JOHNSON: Why did you say that?

LETBY: It's trying to, in that awful situation, have a little bit of -- whatever can be normal and referring to a positive memory; [Baby I] had really enjoyed her first bath. It wasn't meant with any malice. It's -- we still talk to babies, we still treat them as if they're alive. It wasn't joking or malice, it was just trying to reflect on a happier memory.

JOHNSON: Do you understand the difference between life and death?

LETBY: Yes.

JOHNSON: How can you sit there and say, "[Baby I] enjoyed her first bath", when she was dead?

LETBY: No, she'd had her first bath when she was alive, which I'd been present for. That's what I was referring to.

I'm not saying she enjoyed the bath that she was passed away for, I was referring to an earlier bath.

JOHNSON: Oh, which bath was this then?

LETBY: Her first bath that she'd had when she was alive --

JOHNSON: Which one was that?

LETBY: That her mum had done for her.

JOHNSON: When was that?

LETBY: I can't recall a specific date.

JOHNSON: How do you know it was her first one?

LETBY: Because I remember, I was there.

JOHNSON: But how do you know it was her first bath?

LETBY: Because I remember we took photographs. It was a big occasion.

JOHNSON: [Baby I] was at Liverpool Women's Hospital between the 7th and 18 August. How many baths did she have there?

LETBY: I can't comment on that.

JOHNSON: You were getting a thrill out of what you were watching, the grief, the despair that you were watching in that room, weren't you?

LETBY: Absolutely not, no.

JOHNSON: Three weeks or so later, you were taking a picture of the card that you had written out and you were taking that picture in the neonatal unit where [Baby I] had died, weren't you?

LETBY: Yes.

JOHNSON: Because you killed [Baby I], didn't you?

LETBY: No.

MR JOHNSON: My Lord, we're going to move on to [Baby J].

MR JUSTICE GOSS: We'll have our break then. Thank you.

We'll say quarter to, about a 20-minute break, then we'll just have one session, obviously, and finish by 4 o'clock.

(2.23 pm)

(A short break)

(2.45 pm)

(In the absence of the jury)

MR JOHNSON: When the jury comes back, I will give them the agreed versions of the defence statement.

MR JUSTICE GOSS: Thank you, yes.

MR JOHNSON: The paragraph numbers have been maintained by simply putting "not applicable" and "blank" for things that, by agreement, they shouldn't see.

MR JUSTICE GOSS: Thank you.

MR JOHNSON: There's a copy here for your Lordship.

(Handed).

MR JUSTICE GOSS: Thank you.

(In the presence of the jury)

MR JOHNSON: What you're about to receive, ladies and gentlemen, is the defence statement. It's been edited by agreement between us, like the interviews.

Inevitably, when a document is provided -- in this case it was provided in February last year, so significantly before the trial when some of the issues had crystallised. There are things in the original which are not relevant to anything that's in this trial, so we've removed those.

We're going to start with [Baby J] now. Just to remind you, she was born at the Countess on 31 October 2015. She was taken to Alder Hey in Liverpool for surgery on 1 November. She returned to Chester on 10 November and the incident about which we accuse you of trying to kill her happened on 27 November 2015. Okay?

LETBY: Yes.

JOHNSON: I just want to deal with what you say about [Baby J] in your defence statement first of all, which the jury now has. In the original version it's on page 16. For you, ladies and gentlemen, it's paragraph 124 onwards.

So here you say --you start at 124 by saying you did nothing to harm [Baby J]. You then say at 125 that:

"The Countess of Chester NNU didn't appear to be capable of dealing with a baby with stomas, [you] had never seen a baby with stomas at the unit before [Baby J], and when she arrived everyone was apprehensive. None of the nurses had recent experience of stomas. From what [you] could see, the doctors were equally unsure about stomas."

You say that:

"We were reliant on the parents telling us what to do. They had been told what to do by Alder Hey. They were more proficient at changing the stomas than we were."

You say:

"I don't remember [Baby J] being wrapped in a towel and covered in faeces, but if that happened it would have been potentially dangerous for her."

Then at 127:

"On 27 November I was not the allocated nurse for [Baby J]. However, both Mary Griffith and I went into nursery 4 at the same time, having heard the monitor sounding. No one was there. We noticed [Baby J]'s deterioration and seizure and it was after that that she was moved to nursery 2."

Then the final relevant paragraph is 130:

"I didn't decide to keep the handover sheet. It is something that had just remained in my pocket from the end of my shift."

Then, what, found its own way into the Morrisons bag; is that right?

LETBY: No.

JOHNSON: How did it get --

LETBY: I put it there.

JOHNSON: You put it there. When did you do that?

LETBY: I don't know.

JOHNSON: Right. Can we start with tile 140, please. This is the handover to the shift on which you were working on the evening of the 26th into 27 November; all right?

LETBY: Yes.

JOHNSON: We can see, if we just scroll down, please, that [Baby J] began the shift in nursery 4 with another child, both of whom were being looked after by the band 4 nurse Nicola Dennison. You had two children in nursery 3; is that right?

LETBY: Yes.

JOHNSON: There were a couple of children being cared for by a nursery nurse in the transitional care unit. Then Mary Griffith and Caroline Oakley had a child each respectively in nursery 2. In nursery 1, Laura Eagles had two children. We also have two other children for whom we can provide limited details, LW and DM.

LETBY: Yes.

JOHNSON: Are you suggesting that the staffing levels on this shift contributed to any of the incidents that befell [Baby J]?

LETBY: No, it was a very busy shift, but I don't know exactly what happened to [Baby J].

JOHNSON: You're not suggesting it was the staffing levels?

LETBY: No.

JOHNSON: Are you suggesting that the incompetence of any doctor contributed to what happened to [Baby J]?

LETBY: No.

JOHNSON: Any nurse?

LETBY: No.

JOHNSON: Any mistakes that you have been able to identify which contributed?

LETBY: No.

JOHNSON: Do you agree with Mary Griffith's evidence, her assessment of [Baby J], that she was "a very nice baby, a joy to look after"?

LETBY: Yes.

JOHNSON: Do you accept the evidence of [Mother of Baby J] that in the days just before 27 November [Baby J] was well?

LETBY: Yes.

JOHNSON: That she had progressed well?

LETBY: Yes.

JOHNSON: That she was about to go home within a day or two?

LETBY: Yes.

JOHNSON: And that, as a matter of fact, corresponds with a text you sent to your friend [Nurse A] at tile 60, doesn't it? Can we put tile 60 up, please, Mr Murphy?

It's a text sent a few days earlier. I'll read the previous one, it just says:

"How's [Baby J]? Wish I wasn't off, feel so freaking useless."

You reply:

"She's good, I think. In 4, doing well with feeds. Hoping to get home soon."

Is that right?

LETBY: Yes.

JOHNSON: So you gave us quite a lot of evidence about stomas; do you remember that?

LETBY: Yes.

JOHNSON: And how the staff at Chester couldn't cope with the stomas?

LETBY: At times, yes, some of the staff couldn't cope with it, no.

JOHNSON: Well, let's forget about times because [Baby J] was in hospital for quite a long time. Let's deal with the time around her collapses on 27 November. Was there an issue of competence?

LETBY: In terms of her stoma care?

JOHNSON: Yes.

LETBY: Yes. I don't believe that it was necessarily a role that the nursery nurses should have been undertaking.

JOHNSON: Right. And your grounds, please, your reasons for why the nursery nurses shouldn't have been undertaking --

LETBY: It wasn't just mine, it was discussed amongst the nursing staff that, as nursing staff, we had very minimal experience, which would mean that the nursery nurses would have even less experience.

JOHNSON: Which particular staff are you saying had that discussion?

LETBY: I can't name names, but it was widely talked about on the unit.

JOHNSON: So well-known but you can't remember who was talking about it?

LETBY: No.

JOHNSON: You were referred, as part of your evidence, in answer to questions from your own counsel, to tile 46 and I'd just like to put that up, please. You referred to this:

"Minna and I don't think it's right."

And you were suggesting that that was something to do with the stomas; is that right?

LETBY: I can't recall specifically without looking at the context --

JOHNSON: Okay.

LETBY: -- I'm sorry.

JOHNSON: That's a fair comment. Let's go back to the previous text, sent by [Nurse E] to you.

LETBY: Okay, yes, it's about the band 4s doing the re-feeding of the stomas, yes.

JOHNSON: Yes. I don't know whether -- do you remember the evidence of Nicola Dennison, who was the nurse looking after [Baby J] on this shift?

LETBY: Not all details, no.

JOHNSON: It was given on 14 February this year. She was asked whether she had experience of stomas and Broviac lines and she said, "Yes, I did".

LETBY: Yes.

JOHNSON: And she was asked the question:

"Question: Were you comfortable dealing with both of those features?

"Answer: Yes, I was. Can I clarify, though, that we didn't do anything with the Broviac line, it was just present. That was cared for once a week, on Friday, and that was dealt with by registered members of staff."

LETBY: Yes.

JOHNSON: So that was her evidence and it was never suggested to Nicola Dennison, was it, that she in some way lacked competence or experience to deal with a stoma?

LETBY: I don't recall. I just know that in my time at the unit, [Baby J] was the only baby I'd ever seen with a stoma. Within those present years there was no other cases of stomas on the unit.

JOHNSON: But you went further than that, didn't you, on 16 May, this year, in answer to questions from your own counsel?

You were asked these questions:

"Question: Is there a particular band that nursery nurses are at?

"Answer: They're a band 4.

"Question: Can they do intensive care?

"Answer: No, they can't, nor high dependency."

Then this question:

"Question: So far as you understand, should they be involved in handling and recycling stoma bags if that is required?

"Answer: No. And particularly as nursing staff there were unfamiliar, nursery nurses would have been even more so unfamiliar than nursing staff."

And then this question:

"Question: Is there any reason why you had a band 4 dealing with something that presented the problems that stoma bags did?

"Answer: I think the unit was very busy and we had to use staff where we could."

Do you remember that exchange?

LETBY: Yes.

JOHNSON: Were you there suggesting that the reason for Nicola Dennison being put in charge of [Baby J] was because in effect you were short-staffed?

LETBY: No, I wasn't referring to Nicola Dennison specifically, it was generally nursery nurses having [Baby J] over this period of time.

JOHNSON: Well, let's not worry about the period of time overall.

Let's deal with the time that she collapsed.

LETBY: Okay.

JOHNSON: Are you suggesting that there was something wrong with [Baby J] being looked after by a nursery nurse at the time she collapsed?

LETBY: No.

JOHNSON: Well, in the light of that answer, I just want to re-visit something else you said then in answer to your own counsel. You were referred to tile 56. If we can just look at that. Let's do 55 first to give you the context, I'm sorry.

This is [Nurse E] to you:

"Apparently, Eirian said she would have been okay, Jenny..."

That's Jennifer Jones-Key, isn't it?

LETBY: Yes.

JOHNSON: Who was a band?

LETBY: Four.

JOHNSON: "... having [Baby J], but others would need to do recycling, et cetera, but she was one performing it."

And then the next tile, please. You say -- could you read it? They're your words.

LETBY: "Okay, that makes sense. But Jenny was actually doing it? Hmm. Don't know when to stop, do they?"

JOHNSON: Who didn't know when to stop?

LETBY: Sometimes I felt that the nursery nurses would undertake roles that they weren't experienced or trained enough in doing.

JOHNSON: You were complaining here, weren't you, that Jennifer Jones-Key was recycling the stoma?

LETBY: Yes.

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: Do you remember your counsel saying this, having read that tile together with -- we'll deal with all the ones that he dealt with to put this in context. 57 next, please. This is [Nurse E] to you:

"Nope. Laura and [Nurse B] thought it was terrible."

Then 58, you to [Nurse E]. Could you read this?

LETBY: "It's shocking really that they are willing to take the responsibility for things they have no training or experience, et cetera, on. Don't think they appreciate the potential difficulties."

JOHNSON: So you were complaining again, weren't you, in your evidence in answer to your own counsel that this nursery nurse shouldn't have been doing this job?

LETBY: Yes, I don't think it was fair to put that on to a nursery nurse, no.

JOHNSON: Do you remember this was then said to you by your counsel:

"Pausing there, should nursery nurses take on tasks for which they have no training or experience in the case of babies like this?"

Do you remember your answer?

LETBY: No.

JOHNSON: You said no, and you said it was potentially dangerous.

LETBY: That's what I believe, yes.

JOHNSON: So the impression you were giving the court was, first of all, that band 4s were not qualified to do this job; is that right?

LETBY: Not to do their job, to not handle stoma care.

JOHNSON: Yes, and that the hospital was knowingly cutting corners with care by assigning unqualified staff to deal with patients who should have had trained staff?

LETBY: Yes.

JOHNSON: Do you know what the guidelines are for stoma care?

LETBY: No.

JOHNSON: I'm going to show you. Can we put the BAPM guidelines on the screen, please? These are the BAPM guidelines.

LETBY: Okay.

JOHNSON: I'm going to go straight to the point, but if you want to see the context of it we can go back. Can we turn the page, please? And again, please, and again. Right:

"Definition of a special care baby."

What does it say, please?

LETBY: "Special care is provided for babies who require additional care delivered by the neonatal service but do not require either intensive or high dependency care.

"Definition of special care day: any day where a baby does not fulfil the criteria for intensive or high dependency care and requires any of following..."

JOHNSON: And then we cut to "care of stomas". She was a special care baby, [Baby J], wasn't she?

LETBY: Yes.

JOHNSON: What category of nurses deal with special care babies?

LETBY: Nursery nurses, band 4s, but I think you need to appreciate the context in that the unit itself was not familiar with stomas. Other nursery nurses in units where they care for them a lot, yes, I think they could manage that.

JOHNSON: This nursery nurse was familiar with stomas, wasn't she?

LETBY: In her opinion, yes.

JOHNSON: Who are you to contradict her?

LETBY: Because I know that in the 4 years that I was on the unit, I had never seen a stoma before, so she had not dealt with one in 4 years.

JOHNSON: How do you know?

LETBY: I don't know 100% but I certainly can't recall any other baby in that time having a stoma.

JOHNSON: I'm going to make a suggestion to you: that you were deliberately creating the impression with the jury that [Baby J] -- the care provided to [Baby J] was in some way deficient. Do you understand what I mean?

LETBY: Yes.

JOHNSON: Yes.

LETBY: I do think that. I don't think she had a high standard of care.

JOHNSON: Well, what was the deficiency then? Where did it come short?

LETBY: That I don't feel anybody on that unit was overly confident and able to say, "This is what we do with a stoma". We were very much -- we were asking the parents, there were people coming round checking what we were doing or learning from the experience. There was nobody there that clearly said, "This is what we need to do for [Baby J]".

JOHNSON: The nurse who looked after her was sitting where you're sitting now.

LETBY: Yes.

JOHNSON: She said that she was confident and capable to do the job.

LETBY: Well, that's her assessment of herself.

JOHNSON: Why wasn't she challenged?

LETBY: I can't answer that.

JOHNSON: Do you accept the evidence of Dr Gibbs that -- we can take that down, please, Mr Murphy.

Do you accept the evidence of Dr Gibbs that prior to arriving at the Countess on 10 November, [Baby J] had no respiratory problems?

LETBY: Yes.

JOHNSON: Do you accept that she was taking most of her feeds from a bottle by the time of her collapse?

LETBY: Yes.

JOHNSON: Let's go back to tile 140, please. Just before we get to that, the BAPM guidelines were quoted repeatedly to Drs Evans and Bohin when they gave evidence; do you remember?

LETBY: Yes.

JOHNSON: Have you ever read them?

LETBY: Yes.

JOHNSON: Had you ever read that bit about stomas that I have just shown you?

LETBY: I can't recall specifically, no.

JOHNSON: Let's look at tile 140. In answer to questions from your counsel, you said:

"This was a very busy shift and that the doctors were present for quite a lot of the shift."

Do you remember saying that?

LETBY: Yes.

JOHNSON: Let's look at how busy it was at the beginning. You were in nursery 3, weren't you?

LETBY: Yes.

JOHNSON: With two special care babies?

LETBY: I can't say what designation of care they were, but they were in nursery 3, yes.

JOHNSON: There was plenty of space in the intensive care and the -- what's nursery 2 called again?

LETBY: Sorry?

JOHNSON: Nursery 1 is intensive care; yes?

LETBY: Yes.

JOHNSON: Nursery 2 is?

LETBY: High dependency.

JOHNSON: High dependency. So plenty of space in both those if either of your babies had been --

LETBY: Yes.

JOHNSON: -- either intensive or high dependency?

LETBY: Yes.

JOHNSON: So can we take it that your two were special care?

LETBY: Yes.

JOHNSON: [Baby J] and NW being looked after by Nicola Dennison in nursery 4?

LETBY: Yes.

JOHNSON: Vicky Blamire with two children in transitional care?

LETBY: Yes.

JOHNSON: Laura Eagles with two in nursery 1?

LETBY: Yes.

JOHNSON: And your friend Mary Griffith and Caroline Oakley with one each in nursery 2?

LETBY: Yes.

JOHNSON: Together with two children whose location cannot be confirmed?

LETBY: Yes.

JOHNSON: You didn't like being in nursery 3, did you?

LETBY: No, I disagree.

JOHNSON: Were you upset that Laura Eagles was in nursery 1?

LETBY: No.

JOHNSON: Would you rather have been in nursery 1?

LETBY: No.

JOHNSON: Do you accept the evidence of Nicola Dennison that because [Baby J] and NW were due to go home, the policy was to leave them in the room in the dark at night as much as possible?

LETBY: Yes.

JOHNSON: You did have contact with [Baby J] on that shift though, didn't you, before she ever collapsed?

LETBY: I think so from looking at the charts, yes.

JOHNSON: Yes. If we go to tile 149, please. It's also at tile 150, the same information. Do we see that you were, together with your friend Mary Griffith, giving sodium chloride oral supplements to [Baby J]?

LETBY: We would not have given them because oral supplements go in with the milk feed, but yes, Mary and I would have drawn them up, checked them and left them at the cot side with Nicola Dennison to put into the milk, yes --

JOHNSON: I see.

LETBY: -- but we wouldn't have actually given those ourselves.

JOHNSON: I'm going to take things slightly out of order for reasons that will become clear. All right? So far as [Baby J] was concerned on this shift, there were two pairs of incidents; do you remember?

LETBY: Yes.

JOHNSON: The first pair happened in nursery 4 and the second pair happened in nursery 2.

LETBY: Yes.

JOHNSON: Do you remember that? So the first two incidents were documented as having happened at 04.40 and 05.03.

LETBY: Okay.

JOHNSON: Are you with me so far?

LETBY: Yes.

JOHNSON: There were then incidents just either side of 07.00 hours; okay?

LETBY: Mm-hm.

JOHNSON: So the first of those was at 06.56. The second of those was at about 07.24 or thereabouts.

LETBY: Yes.

JOHNSON: You remember all that? Just so that you have the full picture before I start asking you questions about all this, can you go back to the neonatal review, please, and go to page 8.

LETBY: Is this a different folder now?

JOHNSON: It's the black folder, we're on to the black folder.

We'll start at page 1 just to establish what we're looking at. We're looking at event 15.

LETBY: Yes.

JOHNSON: Page 2 gives us the population distribution, which we've already seen in the sequence of events.

LETBY: Yes.

JOHNSON: If you go right to the end of page 8, we have the population distribution at the end of the shift; right?

LETBY: Yes.

JOHNSON: To jog your memory, you will see that [Baby J], first of all, had moved, at the beginning, from nursery 4 to, at the end, nursery 2?

LETBY: Yes.

JOHNSON: But also there, on the right-hand box in the bottom right-hand corner of the page as we look at it on the screen, we have EC and FC, who were twins who had been born at home and were admitted as an emergency at 6.10 and 6.30 respectively.

LETBY: Yes.

JOHNSON: Just to jog everyone's memories, one of those children had a cleft palate; do you remember that?

LETBY: Yes.

JOHNSON: Okay. So that's the bookends of the time parameters, if you like, of the population.

In answer to questions from your own counsel, so far as the first pair of events in nursery 4 were concerned, you said that you had no memory at all of them.

LETBY: That's right.

JOHNSON: Having thought about it, was that answer accurate?

LETBY: Yes.

JOHNSON: Well, I want to give you every opportunity to review what you have said; all right? Do you remember that following the second of the first pair of incidents, [Baby J] was moved from nursery 4 to nursery 2?

LETBY: No. So from my memory, [Baby J] had a seizure in nursery 4 and that was when she moved up, but that was my memory.

JOHNSON: You were involved in an incident in nursery 4, weren't you?

LETBY: Yes.

JOHNSON: And that happened either at 4.40 or at 05.03?

LETBY: I can't say without checking now.

JOHNSON: Well, that was the evidence, you see, that following those two incidents at 04.40 and 05.03, she was moved.

LETBY: Okay.

JOHNSON: So it would follow, wouldn't it, that if you have a memory of anything that happened in nursery 4, it must have been one of those incidents?

LETBY: Or I've been mistaken in my memory and it didn't happen in nursery 4.

JOHNSON: Well, let's start with those events and so if we can go, please, to tile 161 of the sequence of events and keep open also the neonatal review. If we could look at the actual -- this is the nursing note. We see here Nicola Dennison's note, don't we?

LETBY: Yes.

JOHNSON: This ties in actually with the information on the previous tile, 160. If we could just go to that and if we could put the original document up at 160, please.

I'm sure we all remember this. This is Nicola Dennison's handwriting, she told us. Do you remember?

LETBY: Yes.

JOHNSON: It records two events, one at 04.40 and one at 05.03.

Are you with me so far?

LETBY: Yes.

JOHNSON: Those two events appear separately in the sequence of events at tile 160 and 162. Are you with me so far?

LETBY: Yes.

JOHNSON: If we look at the neonatal review, we will see those events at tile 70 -- sorry, line 70 -- sorry, my eyesight's going, line 77 and line 87. Are you with me?

LETBY: Yes.

JOHNSON: Okay. So two events in nursery 4. The reason I suggested to you that it was following that that [Baby J] was transferred is what Dr Verghese told us. Do you remember Dr Verghese?

LETBY: Yes.

JOHNSON: The gentleman with a Northern Irish accent, I think.

LETBY: Yes.

JOHNSON: His notes are at tile 165 of the sequence, please.

I'm not going to examine these notes in detail, but these record the desaturations and the investigations that Dr Verghese undertook and asked for.

LETBY: Yes.

JOHNSON: With me so far?

LETBY: Yes.

JOHNSON: Now, if, with that time frame in mind, we could look at the nursing -- the neonatal review, please. The first desaturation was at, if we look at line 77, 04.40.

LETBY: Yes.

JOHNSON: We can see that between 04.47, which is line 79, and line 81, you were making a nursing note entry on a computer for the child LF, who was in nursery 3.

LETBY: Yes.

JOHNSON: So in other words, the desaturation happened 7 minutes before you started to make the nursing note.

LETBY: Yes.

JOHNSON: And the second desaturation happened about 4 minutes after you made that note.

LETBY: Yes.

JOHNSON: And you have a memory because you have told us that you and Mary Griffith went into nursery 4.

LETBY: Yes, but [Baby J] was fitting at that point from my memory.

It wasn't a desat, she was fitting.

JOHNSON: Well, at tile 177 -- we'll start at 175, actually, to get the context. [Nurse E], at 06.24, sent you a text:

"Good night?"

Do you remember that?

LETBY: Yes.

JOHNSON: And you replied at tile 177 at 06.28. Could you read it for us, please?

LETBY: "No. [Baby J] in 2 screened. Had profound desats and just got 32-week twins born at home, one with cleft lip, and only 5 staff."

JOHNSON: So it follows, doesn't it, that by 06.28, as I suggested to you earlier, [Baby J] had been transferred to nursery 2?

LETBY: Yes.

JOHNSON: Otherwise you'd be looking into the future, wouldn't you?

LETBY: Yes.

JOHNSON: That's precisely the language Dr Verghese used, "profound desats". Did you copy his language from the medical notes?

LETBY: No.

JOHNSON: Tile 165. Can we just go back, please? "Two profound desats." Do you see that at the beginning of his note?

LETBY: Yes.

JOHNSON: At the time you were sending that text to [Nurse E], that's tile 177, the one we just looked at, I'm suggesting that you were in nursery 2 with [Baby J].

LETBY: Okay.

JOHNSON: What do you say?

LETBY: I can't recall where I was when I was sending that message.

JOHNSON: We also see in tile 177 the reference to the twins, don't we?

LETBY: Yes.

JOHNSON: Who we have already established were admitted, if we're looking at the neonatal review at line 114, the first one at 06.10?

LETBY: Yes.

JOHNSON: And the second at line 131 at 06.30?

LETBY: Yes.

JOHNSON: As you say in that text at tile 177, only five staff.

LETBY: Yes.

JOHNSON: So it was all hands to the pump, wasn't it --

LETBY: Yes.

JOHNSON: -- for the twins?

LETBY: Yes.

JOHNSON: Yes. So resources in the neonatal unit at about 6 am on 27 November would all have been concentrated in nursery 1; do you agree?

LETBY: Not all efforts, but yes, a lot of it, yes.

JOHNSON: Yes. But you were not part of that effort, were you?

LETBY: I can't say from my memory, no.

JOHNSON: Well, you don't need to rely on your memory because if we go to the texts we will see. The next one, please.

This is, bearing in mind first twin in at 6.10, second twin in at 6.30, you at 6.29 receiving a message from [Nurse E] saying:

"Oh shit. Five staff on today, you mean?"

And you replied?

LETBY: "Yeah, 5 today."

JOHNSON: And you sent that at what time, please?

LETBY: 06.31.

JOHNSON: So you were not in nursery 1, were you?

LETBY: No.

JOHNSON: Just pausing there for a second, how many of the nursing staff would it have taken to look after two babies that were already in nursery 1 and these additional two who were an emergency admission?

LETBY: Ideally they would have all been one-to-one.

JOHNSON: Yes. So can we say at least three?

LETBY: No, because that wouldn't have been manageable, we didn't have the staffing for that and I think Laura had two babies in nursery 1 to start with.

JOHNSON: Yes, absolutely. So Laura had two to start with and the other qualified staff were Mary Griffith, Caroline Oakley and you?

LETBY: Yes.

JOHNSON: The children in nurseries 3 and 4 were special care?

LETBY: Yes.

JOHNSON: There were two nursery nurses working that night, Nicola Dennison and Vicky Blamire?

LETBY: Yes.

JOHNSON: And they could have looked after the kids in nurseries 3 and 4?

LETBY: Potentially, yes.

JOHNSON: Yes. And so if each of the twins who were admitted as an emergency had a nurse each, that would leave only you left of the qualified staff to look after the children in nursery 2, including [Baby J], wouldn't it?

LETBY: No, because Mary wasn't intensive care trained, so she wouldn't have been able to go into nursery 1. It would have been me if there was a third member in 1.

JOHNSON: We know you weren't there, don't we?

LETBY: No -- well, yes -- so, yes, but Mary wouldn't have been in there either.

JOHNSON: We know you weren't because you were texting your mate, weren't you?

LETBY: Yes.

JOHNSON: If we carry on, we will see what I'm talking about. So this is a message sent by you to [Nurse E] at 06.31.

Tile 180 next, please. She says within the same minute:

"We closed again?"

And then at 06.40, 185:

"Jesus! This will be an interesting allocation then. Are we closed?"

Then what did you say at 186?

LETBY: "Yeah, closed, trying to get someone in. Cleft baby being tubed I think."

JOHNSON: Let's just look at where you were at this point; okay?

Let's go back to the neonatal review, please. Can we go to the bottom of page 4. This is just before your text conversation with your friend [Nurse E] started. So at 6.15, in other words after [Baby J] had been moved to nursery 2, where were you?

LETBY: With baby FB.

JOHNSON: Yes. Nursery 2?

LETBY: Right.

JOHNSON: Well, do you agree?

LETBY: Yes.

JOHNSON: Let's look at where you were at 06.22 over the page, lines 122 and 123. With baby KB?

LETBY: Yes.

JOHNSON: Who was where, please?

LETBY: Nursery 2.

JOHNSON: Yes. So you were in nursery 2, weren't you --

LETBY: Yes.

JOHNSON: -- when the emergency was unfolding relating to the twins who had been admitted?

LETBY: Yes.

JOHNSON: There would have been lots of distractions, wouldn't there, with these two emergencies?

LETBY: I don't know what you're implying.

JOHNSON: I'm implying, I'll make it clear, that the medical staff would have been having their attentions focused on the twins; do you agree?

LETBY: Yes.

JOHNSON: And that any nursing help that could be spared would also have been focused on those twins?

LETBY: If needed, yes.

JOHNSON: Yes. Well, such was the emergency that Dr Gibbs came in early, didn't he?

LETBY: Yes.

JOHNSON: Yes. So do you accept that a lot of help was needed?

LETBY: I can't say without looking at the twins specifically, but it would be normal practice to get a consultant in when

we've just had two new admissions and we've only got a registrar, yes.

JOHNSON: So if there's any doubt about it, if we go back to the sequence of events, please, at tiles 181 and 182, there we see Dr Gibbs' arrival, don't we, at 06.33 and 06.34?

LETBY: Yes, I believe he'd been in earlier in the night as well, I think.

JOHNSON: At 185 we had the message that we just saw from [Nurse E] about it being an interesting allocation, and then your response at 186, please. Would you read it?

LETBY: "Yeah, closed. Trying to get someone in. Cleft baby being tubed I think."

JOHNSON: Yes. So were you keeping an eye on what was going on in nursery 1?

LETBY: I can't say I was keeping an eye, but I'm aware that potentially the cleft baby is being tubed, yes.

JOHNSON: She responded:

"What a nightmare. What a bloody nightmare."

And 187 and then:

"Please tell me Ailsa's not in charge today."

At 188. Then you responded what at 189?

LETBY: "You are having twins until we can find someone."

JOHNSON: Yes. And then at the same time, 190:

"And sorry, but yes."

And then she asked you:

"What, the one that's being vented?"

And then you responded what, please?

LETBY: "They are holding off at mo."

JOHNSON: And?

LETBY: "It's all a bit tits-up".

JOHNSON: Then your text messaging stopped, didn't it?

LETBY: I can't say. I don't know.

JOHNSON: Well, it did. If we move on to the next tile. That's [Nurse E] to you at 194, 06.50?

LETBY: Yes.

JOHNSON: And the next one, 195?

LETBY: Yes.

JOHNSON: 06.52, her to you, but you were not responding?

LETBY: No.

JOHNSON: Because you were in nursery 2 sabotaging [Baby J], weren't you?

LETBY: No, I was not, no.

JOHNSON: Well, we know from Dr Gibbs' note, if we can go to it, please, at 197, so it's two tiles on, that it was -- if we go to the... The first desaturation was at 06.56.

LETBY: Yes.

JOHNSON: In other words, 7 minutes after the last text you'd sent your mate [Nurse E] saying, "It's all a bit tits-up"?

LETBY: Yes.

JOHNSON: Just look at that half an hour between 6.28 and 6.56, which is the time of the first desaturation. Can we go back to the neonatal review, please? Can we look at the top of page 5 of 8? Were you recorded as doing anything in that half hour?

LETBY: No.

JOHNSON: No. We know that you were texting your friend, [Nurse E].

LETBY: Yes.

JOHNSON: And we know that you were in nursery 2 at line 123 at 06.23.

LETBY: Yes.

JOHNSON: And we know from the records that the twins were -- one had just been admitted and one was about to be admitted.

LETBY: Yes.

JOHNSON: So things were very busy, weren't they?

LETBY: Yes.

JOHNSON: But despite the fact that these twins were posing an emergency, Dr Gibbs' attentions were deflected to deal with [Baby J], weren't they?

LETBY: Yes.

JOHNSON: If we just go back to his note, please. What we see in his distinctive writing is his record of the first desaturation to what he says are "unrecordable levels"

at 06.56 and 07.24.

LETBY: Yes.

JOHNSON: Don't we? Both associated with clenching of hands, stiff limbs and, on two occasions, eyes deviated to the left.

LETBY: Yes.

JOHNSON: Took 10 minutes to settle on the first occasion and 3 to 4 on the second occasion. This was your doing, wasn't it?

LETBY: No, it was not.

JOHNSON: This was a serious emergency for [Baby J], wasn't it?

LETBY: Yes, it was, yes.

JOHNSON: Yes. So serious that it diverted Dr Gibbs from the emergency admissions?

LETBY: Yes.

JOHNSON: Happening to a baby in nursery 2 and you were in nursery 2?

LETBY: I don't know that I was there at that exact moment in time.

JOHNSON: You weren't anywhere else, were you?

LETBY: We can't say that. We don't know.

JOHNSON: Well, you were there, you tell us.

LETBY: I did not do anything to [Baby J].

MR MYERS: She just said she couldn't remember, my Lord.

MR JUSTICE GOSS: You did refer earlier to being aware of [Baby J] fitting.

LETBY: Yes.

MR JUSTICE GOSS: Can you describe the circumstances of how you came to be aware of [Baby J] fitting?

LETBY: My memory was that Mary Griffith and I heard the monitor in nursery 4 and when we went down to nursery 4 that's when we found [Baby J] fitting. So my recollection was the first seizure was in nursery 4, but I accept that may be a misrecollection and she may have been in nursery 2 by that point.

MR JOHNSON: Mr Myers just intervened on your behalf saying that you had said you couldn't remember. You didn't say that, did you?

LETBY: What about?

JOHNSON: Couldn't remember where you were. He suggested an answer to you that you hadn't actually given. What you said is, "We can't say that, we don't know".

LETBY: Right.

JOHNSON: Well, which is it?

LETBY: I can't say where -- I don't know if I was with [Baby J] at that time. Is that what you're referring to?

JOHNSON: Oh, I'm suggesting you were with [Baby J] because it was you who sabotaged her.

LETBY: No, I wasn't, no.

JOHNSON: And you took your opportunity when all the other resources of the NNU were concentrated on the two twins that had been admitted as an emergency.

LETBY: No.

JOHNSON: Do you remember Dr Gibbs and Dr Brearey saying that there was no explanation for what had happened --

LETBY: Yes.

JOHNSON: -- to [Baby J]? Her blood sugars were normal.

LETBY: Yes.

JOHNSON: Her electrolytes were normal. She had no bacterial infection. Do you remember?

LETBY: Yes.

JOHNSON: Her blood test was normal. Her brain scan, normal.

X-ray, normal. No rise in CRP or white cell count. The only abnormal thing was you sabotaging her, wasn't it?

LETBY: No.

JOHNSON: Do you remember in your evidence you were taken to the following night shift, 28 November?

LETBY: I don't recall that, no.

JOHNSON: No? Well, I'll remind you. Tile 324, please. Just the analyst's transcription, please, of 324.

This was the following night; okay?

LETBY: Yes.

JOHNSON: This is your note.

LETBY: Yes.

JOHNSON: And it was pointed out to you that you had had [Baby J] on the following night shift and nothing had happened to her.

LETBY: Yes.

JOHNSON: Do you remember that?

LETBY: Yes.

JOHNSON: Was the implication that if you had done something to her the previous day, you would have done something on this shift and because you didn't it shows you didn't do something the previous night?

LETBY: Sorry, can you rephrase that?

JOHNSON: Yes. Was the purpose of your answers to suggest that because nothing happened to [Baby J] on this shift, if you had been trying to do something to her on the previous shift you would have done something on this one?

LETBY: I don't quite understand, but what I'm saying here is that I've looked after [Baby J] and nothing --

JOHNSON: Yes, nothing happened to her?

LETBY: Yes.

JOHNSON: There is a detail that you weren't pointed to, though, and I'd like to go to the original note that's behind that, please. It's in the bottom right-hand corner.

You see there -- who else was on the unit that night with [Baby J] that nothing happened to her?

LETBY: Parents.

JOHNSON: Yes. Can we just go to your interviews because there's one matter I'd like to ask you about, please. It's the second interview bundle, please, interview bundle 2. Do you remember me asking you about being in nursery 2 before?

LETBY: Yes.

JOHNSON: Let's just look at the bottom of [Initials of Baby J] 2, which is the second page of the first interview, please.

If you read up from the bottom of the page about three or four inches there's a question:

"Question: So you weren't her designated nurse. Were you required -- required to treat her other than obviously from the two pages [as read]? It says. And you say:

"Answer: From my memory, there's an occasion when [Baby J] was transferred into my nursery."

LETBY: Yes.

JOHNSON: And then:

"Answer: I'm not sure specifically on the date, but there was a date that she was down in nursery 4 and she was moved up into nursery 2."

LETBY: Yes.

JOHNSON: And that is your recollection of being in nursery 2 when these events were happening, isn't it?

LETBY: I don't understand.

JOHNSON: Yes, that's you saying to the police that you were in nursery -- I'm suggesting that's you saying to the police you were in nursery 2 when these events happened.

LETBY: Yes.

JOHNSON: Yes. And that was the truth, wasn't it?

LETBY: I -- I don't know.

JOHNSON: Well, I'm going to move on to [Baby K]. I'm sure that I won't complete that by 4 o'clock, but I'm quite prepared to make a start.

MR JUSTICE GOSS: I think we will make a start, please.

MR JOHNSON: Yes, of course.

Can we start with the defence statement, please.

Put the interviews away, please. If we start at paragraph 131, if you would. You start, as you do in every case, by saying that you did nothing to hurt [Baby K]. You then say:

"I question the extent to which [Baby K] was properly intubated at the Countess of Chester. I do not accept that this was properly done, nor do I accept that the post-delivery care of [Baby K] was to the optimal or appropriate standard."

We'll come back to that, so I'd like -- probably not today, but I'd like you to think about the deficiencies if you would.

133:

"I do not recall the events of 17 February. I've no recollection of Ravi Jayaram coming into nursery 1 when I was there by myself. I do not recall saying that [Baby K] had just started to desaturate. I believe that [Baby K] was capable of moving and dislodging the tube however it had been fitted. Furthermore, I question whether the tube might have been blocked. I had done nothing to interfere with it, nor did I interfere with the alarm.

"I don't believe that anyone on the unit had sufficient experience to look after a 25-week gestational baby, certainly none of the nursing staff did. I don't believe that the care

[Baby K] received at the Countess of Chester was appropriate or optimal. This is only one example of how the unit was taking babies it should not have been taking, given the limits on the level of care it was capable of delivering and the staffing levels at the material times."

Okay?

LETBY: Yes.

JOHNSON: Is that, first of all, what the document says?

LETBY: Yes.

JOHNSON: Is it accurate?

LETBY: Yes.

JOHNSON: Just so that I understand and the jury can understand, are you saying that you were not with [Baby K] at 3.50 when Dr Jayaram says you were standing over her as she desaturated?

LETBY: I have no recollection of that.

JOHNSON: Well, I understand that answer, of course, but there is a difference between the question and your answer. Are you saying that Dr Jayaram has made that up? Are you saying it is untrue?

LETBY: I don't think I can comment on whether he's made that up. I have no memory of myself being in there at that time having a conversation with Dr Jayaram. I can't go beyond that.

JOHNSON: Right. Do you dispute it?

LETBY: I think that's difficult when I have no memory. I don't recall any of that happening, no, and I think I would remember if I had been present.

JOHNSON: Have you changed your position since I started to ask you questions about this?

LETBY: No.

JOHNSON: Well, let's start with the reasons why [Baby K] was born at the Countess of Chester. Do you remember telling the jury that you didn't understand why she was born there?

LETBY: Yes.

JOHNSON: Do you not remember [Mother of Baby K]'s statement, which was read to the jury?

LETBY: Yes.

JOHNSON: And what did she say?

LETBY: That she hadn't been fit for transfer to another hospital.

JOHNSON: Yes. That she went in with pains, went into labour, there was no bed available at Arrowe Park, the only bed that was available was in Bristol, and because she was 4 centimetres dilated it was too risky for her to be transferred. Do you accept that's a good reason for [Baby K] to have been born at the Countess?

LETBY: That's an obstetric decision. I don't feel that's for me to comment on.

JOHNSON: But do you understand the reason why she was born at the Countess of Chester?

LETBY: Yes.

JOHNSON: Why did you tell the jury that you didn't understand why she'd been born at the Countess of Chester?

LETBY: Because I don't know why more effort wasn't made to have found a bed somewhere for her.

JOHNSON: Well, I'll suggest an alternative reason to you for your comment: you have persistently tried to create the impression that Chester were taking on babies for whom they were not capable of caring for and that is the reason why they collapsed.

LETBY: Yes.

JOHNSON: Is that the reason why you said to the jury that you didn't understand why [Baby K] was born at Chester?

LETBY: I don't understand why she was born at Chester.

JOHNSON: Please answer the question: is it to try and bolster your defence that you said to the jury that you didn't understand the reason why [Baby K] was born at Chester?

LETBY: No.

JOHNSON: Well, what was the reason you said it if you did understand why she was born at Chester?

LETBY: It's two separate -- I understand why she was born there, but I don't necessarily agree that that was the right thing or that it had to be that way.

JOHNSON: All right. So you don't accept the agreed evidence that to have transferred her was a greater risk than to have delivered [Mother of Baby K] of [Baby K]?

LETBY: That's an obstetric thing, I can't comment on that.

JOHNSON: But you're --

LETBY: I'm sorry, I don't really understand what you're asking me.

JOHNSON: All right. There were three extubations of [Baby K], weren't there?

LETBY: Yes, I believe so.

JOHNSON: Yes. There's the first one at 03.50?

LETBY: Yes.

JOHNSON: The second one at 06.15; yes?

LETBY: Yes.

JOHNSON: And the third one at 07.35?

LETBY: Okay.

JOHNSON: Well, do you accept that, first of all?

LETBY: Yes. I can't see the information in front of me now, but if that's the times agreed, then yes. Sorry, I can't recall from memory exactly what happened at what time.

JOHNSON: All right. Well, that's a perfectly reasonable response. The easiest way to look at this is to go to the neonatal review just for you to satisfy yourself that I'm not leading you up the garden path. This is the [Baby K] neonatal review, so on the front it should say "Event 16: [Baby K]".

LETBY: Yes.

JOHNSON: If you go to page 5 of 9, go to line 122, that's the first event, the first extubation or misplaced tube.

LETBY: Yes.

JOHNSON: The second is at 06.15, which is on page 7 of 9 and it is line 209. Have you got that?

LETBY: It's not in a black line?

JOHNSON: It isn't in a black line, but if you read across -- I was trying to do it a quick way because we're getting to the end of the day.

LETBY: I see it now.

JOHNSON: If you don't follow, I'd much rather you said so than go along with things that I suggest to you.

The third desaturation is at 07.35 which actually I don't think, as a matter of fact, is recorded in this document, so that's not enormously helpful. I'll get the tile number. We'll deal with that as we go along.

I want to make sure I understand your case. So far as the first desaturation is concerned, you just have no recollection of it; is that right?

LETBY: That's right.

JOHNSON: Therefore you are not prepared to accept what Dr Jayaram says about it?

LETBY: No.

JOHNSON: So far as the second desaturation at 06.15 is concerned, what do you say about that?

LETBY: I don't recall being a part of that.

JOHNSON: Right. What about the third event that [Nurse B] told us about at 07.35?

LETBY: Again, I don't have any memory.

MR JOHNSON: All right. I want to deal with one of these before we break if I may, my Lord.

MR JUSTICE GOSS: Yes.

MR JOHNSON: I'm not going to deal with these in chronological order; all right?

LETBY: Okay.

JOHNSON: Because what I want to do is ask you questions about Dr Jayaram's evidence after we've looked at the other two events; do you understand?

LETBY: Yes.

JOHNSON: So we have desat 1 at 03.50, 2 at 06.15, 3 at 07.35?

LETBY: Yes.

JOHNSON: Are you with me so far?

LETBY: Yes.

JOHNSON: What I'd like to do is to concentrate, first of all, on the second desaturation; okay?

LETBY: Okay.

JOHNSON: Just before we get to that and in case I forget to ask you, can we start with tile 21, please. There we have the staffing position at the beginning of the shift. We know, of course, that [Baby K] went into nursery 1 --

LETBY: Yes.

JOHNSON: -- during the shift having been born at 02.12. But we see that before she went in there, there were two children in nursery 1.

LETBY: Yes.

JOHNSON: Three in nursery 2 in respect of whom you were the designated nurse for two?

LETBY: Yes.

JOHNSON: Three in 3?

LETBY: Yes.

JOHNSON: And three in 4?

LETBY: Yes.

JOHNSON: Are you suggesting that staffing levels caused or contributed to any of the incidents involving [Baby K] on this shift?

LETBY: I don't know what happened to [Baby K].

JOHNSON: So do we take it then that you do not suggest --

LETBY: No.

JOHNSON: -- staffing levels made a contribution? Are you suggesting that any specific mistakes made by the medical staff made a contribution?

LETBY: I potentially feel that the ET tubes were not secured appropriately --

JOHNSON: Right.

LETBY: -- and that [Baby K] was not sedated.

JOHNSON: Okay, we'll deal with that. What about nursing mistakes?

LETBY: Again, that would coincide with the security of the tubes and how [Baby K] was being managed.

JOHNSON: So, what, is the securing of an ET tube a nursing or a medical responsibility?

LETBY: The initial securing would be medical and then ongoing care for the tube would be nursing.

JOHNSON: Right. Okay, well, let's deal, as I say, out of order with the second desaturation; okay?

LETBY: Okay.

JOHNSON: I want to start, if we can, with tile 106. This is just to remind you of what was going on. All right? I'm not going to ask you questions about this specifically.

Just looking at this transposition of Dr Jayaram's handwritten notes will suffice, but if you want to look at the notes, you say so. Okay?

LETBY: Yes.

JOHNSON: There we can see Dr Jayaram recording that at 06.15 [Baby K] began to have lower sats and IV down to 2.5.

Then capillary gas, the saline bolus, and the fact that the tube had to be pulled back to 6 centimetres.

LETBY: Yes.

JOHNSON: Extubated, bagged and then re-intubated.

LETBY: Yes.

JOHNSON: Just before that desaturation, you were with [Baby K], weren't you?

LETBY: I don't recall.

JOHNSON: Let's look at tile 104 then, the admission record for [Baby K]. She collapsed or there was a problem with the tube, to put it a different way, at 06.15 according to Dr Jayaram.

LETBY: Yes.

JOHNSON: You admitted her to the NNU at 06.04 to 06.10, didn't you?

LETBY: Yes, which is a paper exercise, yes.

JOHNSON: It is a paper exercise. What paper?

LETBY: Sorry?

JOHNSON: What paper?

LETBY: It's an admission on the Meditech system and on to the BadgerNet system.

JOHNSON: Yes. What information did you record?

LETBY: It would be her birth details.

JOHNSON: Yes. What else?

LETBY: Um... I can't recall the whole admission form.

JOHNSON: How many children have you admitted?

LETBY: A lot.

JOHNSON: Right. What do you record on an admission form?

LETBY: The pregnancy history, the delivery, how they've been managed at delivery, um... ongoing care plans, parent details.

JOHNSON: Any other detail?

LETBY: I -- I honestly can't recall the whole of the sheet right now.

JOHNSON: Vital signs?

LETBY: On admission, yes.

JOHNSON: Where do you get that information from?

LETBY: The paper admission sheet.

JOHNSON: Where do you get that from?

LETBY: The notes.

JOHNSON: Where do you get them from?

LETBY: The baby's cot side.

JOHNSON: Thank you. We got there in the end. So you had to go to [Baby K]'s cot side in order to carry this exercise out, didn't you?

LETBY: No, the notes would have been taken to the computer.

JOHNSON: By?

LETBY: Myself.

JOHNSON: And where did you get them from?

LETBY: From the cot side.

JOHNSON: Is this a bit like the handover sheets coming home with you?

LETBY: No, I'd have needed to take the notes to the computer.

JOHNSON: Yes, absolutely. And you get them from the cot side?

LETBY: Yes.

JOHNSON: And where do you go after you have got the notes?

LETBY: To a computer. I don't know which computer I used.

JOHNSON: I think you know where I'm going, but we'll dance the dance if you want to --

MR MYERS: My Lord, again, it's comments like, "I think you know where I'm going, but we'll dance the dance", and I don't want to get up and object to them every time, but that's just a comment. It can be quite belittling and can potentially be a little

overbearing and I'd be grateful if comments like that aren't made and just continue with the questioning.

MR JUSTICE GOSS: Yes, I think refrain from the comments, Mr Johnson, and just get to the point, please.

MR JOHNSON: Yes.

You took them back to the cot side, didn't you?

LETBY: After completing the form, yes.

JOHNSON: Yes, thank you. So let's look at the form, please.

This is timed by the computer, isn't it?

LETBY: Yes.

JOHNSON: You created the note at 06.04?

LETBY: Yes.

JOHNSON: You closed the note at 06.10?

LETBY: Yes.

JOHNSON: You have the date of delivery, time of delivery, all taken from the paper notes that you had retrieved from the cot side?

LETBY: So it would come from the paper admission sheet, the observation charts, anything the doctors had written in the medical notes, and any notes from mum as well.

JOHNSON: Yes, taken from the cot side?

LETBY: If that's where they were beforehand, yes.

JOHNSON: Well, you've just told us that they were, haven't you?

LETBY: Yes.

JOHNSON: Yes, thank you. So for example, observations: temperature 38.5, respiration 50, mean blood pressure 45, saturations of oxygen 93% in 80% oxygen; all taken from the paper charts from the cot side?

LETBY: I believe so, yes.

JOHNSON: Yes. Can we scroll down, please? We have other details as well, don't we?

LETBY: Yes.

JOHNSON: Can we just scroll right down, please, on to next page?

What does it say in the top left-hand corner of that page?

LETBY: "Neonate care of endotracheal tube."

JOHNSON: Yes. And?

LETBY: "Patient has an artificial airway..."

JOHNSON: Yes. Continue, please.

LETBY: "... so secure ETT to taped bonnet."

JOHNSON: And then?

LETBY: "Retie tapes with 2 nurses in case of accidental extubation."

JOHNSON: What's all that a reference to?

LETBY: That's an automatic care plan that's produced once you put that a baby's ventilated.

JOHNSON: Yes. Are you looking at this as you are creating it?

LETBY: No, I'm not checking the ET tube, no. This is a generic thing of when we admit a baby and they've got needs such as oxygen, medications, this is an automatic thing that would come up on to the screen.

JOHNSON: It self-populates, is that a reasonable way of putting it?

LETBY: Yes.

JOHNSON: But you were at [Baby K]'s cot side a minute or two before she desaturated for the second time, weren't you?

LETBY: I agree I've got the notes at some point, yes. I can't be specific exactly when.

JOHNSON: Well, if you took them straight back just after you completed this form, that would put you at the cot side --

LETBY: Well, I don't know when I took them back. That's impossible to say.

JOHNSON: What would be your normal practice?

LETBY: That would be completely dependent on what was going on on the shift.

JOHNSON: You help us then. Go to the neonatal review, please.

Can we go to page 7 of 9? At 6 o'clock when you were filling this in, you recorded yourself as being with AR, didn't you?

LETBY: Yes.

JOHNSON: How do those events live together?

LETBY: As we've discussed before, the 6 o'clock for observation charts or fluids is an approximation, it doesn't mean it was done exactly at that moment in time.

JOHNSON: No, but the timing recorded by the nursing notes is not an approximation?

LETBY: No, that would be accurate, yes.

JOHNSON: Yes. Another case of you cooking the notes, isn't it --

LETBY: No.

JOHNSON: -- to put yourself somewhere else?

LETBY: No.

JOHNSON: Because you moved [Baby K]'s tube when you took those notes back to her cot side, didn't you?

LETBY: No, I did not, no.

MR JOHNSON: We'll come to the third event and then go back to Dr Jayaram on Monday, please, my Lord.

MR JUSTICE GOSS: Yes. Thank you, members of the jury.

10.30 on Monday morning then, please. Do remember your responsibilities as jurors not to communicate with anyone about anyone or anything to do with this case in any way and only discuss the case when you are all together in the earshot -- everyone is in the earshot of the other 11. If you're the speaker, the other 11 can hear you, which is basically (inaudible), as I understand it, when you're all there. Thank you very much indeed. 10.30 Monday morning.

(4.05 pm)

(The court adjourned until 10.30 am on Monday, 5 June 2023)